Nintedanib + docetaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination · docetaxel Day 1 and steroid premedication

Active medicines: Docetaxel

Emetogenic risk: Low parenteral

Medicine	Dose / route	When
Dexamethasone premedication	8 mg orally twice daily for 3 days	Start the day before docetaxel; at least 3 doses before treatment Source docetaxel premedication. If missed, consultant may consider 20 mgIV immediately before; not an automatic substitution. Do not duplicate with antiemetic steroid.

Additional phase-specific support

- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) prophylaxis; determine need and timing from individual infection risk and local policy.
- Low-risk antiemetic reference: dexamethasone 8 mg on Day 1 and as-needed rescue according to NCIS/local policy. The docetaxel premedication is already a steroid course: reconcile into one prescription.
- Monitor full blood count (FBC) before each cycle and around nadir; fever needs prompt assessment.
- Hypersensitivity, fluid-retention and extravasation precautions; assess neuropathy, mouth and skin toxicity.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00372 v6 · p4 · NCCP 00372 v 6; NCIS antiemetic guidance

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Combination · nintedanib Days 2–21

Active medicines: Nintedanib

Emetogenic risk: Minimal-to-low oral

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine oral antiemetic prophylaxis in minimal-to-low pathway; prescribe as-needed treatment after interaction/QT/constipation review.
- Give an early diarrhoea action plan with oral hydration and prescribed anti-diarrhoeal treatment; ongoing/severe symptoms need assessment and possible interruption.
- Nintedanib is not taken on docetaxel Day 1. No double dose after a missed capsule.
- Review P-gp interactions, liver tests, blood pressure (BP), bleeding, renal status, surgery/wound healing and contraception.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00372 v6 · p4 · NCCP oral antiemetic pathway

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required